

COOK COUNTY CHILDREN'S REHABILITATION SERVICES

Physical Therapy

1859 W. Taylor Street, Suite 1B, Chicago, Illinois 60612

Phone: (312) 555-4700 | Fax: (312) 555-4709

PHYSICAL THERAPY EVALUATION

Patient: Marcus D. Whitfield **MRN:** PED-2025-04471

DOB: 03/03/2016 **Age:** 9 years **Sex:** Male

Date of Evaluation: 02/20/2026

Referring Provider: Mei Liang, MD — Developmental Pediatrics

Evaluating Therapist: Karen Nowak, DPT

REASON FOR REFERRAL

Bilateral toe walking observed during developmental pediatrics visit (09/15/2025). Referral for gait evaluation and possible ankle-foot orthosis (AFO) assessment.

HISTORY

Marcus is a 9-year-old male with ASD Level 2, ADHD, FASD, and multiple medical comorbidities referred for evaluation of persistent bilateral toe walking. Per caregiver report, toe walking has been present since early childhood and is most prominent during periods of sensory overstimulation or anxiety. He does not use any assistive devices. He is ambulatory and independent for age-appropriate mobility but demonstrates an atypical gait pattern.

EXAMINATION

Observation: Bilateral toe walking observed during ambulation in the clinic hallway. Pattern is persistent but not fixed — Marcus can be cued to walk with heels down, but reverts to toe walking within 5 to 10 steps. Stance phase shows minimal heel contact. Swing phase unremarkable.

Range of Motion: Ankle dorsiflexion with knee extended: R 5 degrees, L 5 degrees (mildly limited; normal ≥ 10 degrees). Ankle dorsiflexion with knee flexed: R 10 degrees, L 12 degrees (suggesting gastrocnemius tightness rather than fixed equinus). All other lower extremity ROM within functional limits.

Strength: Grossly 4/5 bilateral lower extremities. Core stability reduced — difficulty maintaining single-leg stance > 5 seconds bilaterally.

Balance: Romberg negative. Tandem walking with difficulty — multiple step-outs.

ASSESSMENT

Bilateral idiopathic toe walking in the setting of ASD and FASD, with mild gastrocnemius tightness contributing to limited ankle dorsiflexion. Pattern is habitual/sensory-driven rather than fixed contracture at this stage.

PLAN

- AFO referral placed for bilateral hinged ankle-foot orthoses to promote heel-strike pattern and prevent progression of gastrocnemius contracture. AFO fitting appointment to be scheduled.
- Home stretching program provided to caregiver: gastrocnemius/soleus stretching, wall push-ups for ankle dorsiflexion, 2 times daily.
- Physical therapy 1x/week for 8 weeks for gait training, strengthening, and balance.
- Re-evaluate in 8 weeks. If no improvement with conservative management, consider referral to pediatric orthopedics for serial casting evaluation.

Karen Nowak, DPT

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Electronically signed: 02/20/2026

Dictated and finalized on 02/20/2026